

**SUBJECTIVE:**

- Patient presents with a chief complaint of headache and double vision.
- Headache is described as dull, located in the back of the head, and began today. Patient reports the headache is worsening and expresses a desire for ibuprofen.
- Visual disturbance started yesterday, affecting the right eye only, described as "double vision," "ghosted image," "blurry," and "harder to distinguish," with objects "blending more." The double vision is noted as "oblique." Patient also reported sensitivity to "light," which could indicate photophobia or light sensitivity.
- Patient denies other visual disturbances, eye pain with movement, or other neurological symptoms.

**OBJECTIVE:**

- Expected physical exam findings include:
- General: Patient appears uncomfortable or distressed.
- Neurological: Alert and oriented. Cranial nerve examination with specific attention to visual fields and extraocular movements.
- Ophthalmological: Reduced visual acuity in the right eye, especially without pinhole correction. External eye exam, pupillary exam, slit lamp examination to assess cornea, iris, and lens, and funduscopy to evaluate the retina and optic nerve.
- Vital signs: Unavailable.

**ASSESSMENT:**

- Primary Diagnosis: Monocular Diplopia, Right Eye, with associated Headache.
- Differential Diagnoses for Monocular Diplopia:
- Ocular surface issues: Dry eye, irregular astigmatism, corneal opacities/scars, keratoconus.
- Lens abnormalities: Cataract, lens subluxation/dislocation.
- Retinal conditions: Epiretinal membrane, macular edema.
- Other uncorrected refractive errors.
- Differential Diagnoses for Headache:
- Tension-type headache, potentially exacerbated by eye strain.
- Primary headache disorder (e.g., migraine variant, less likely given dull quality).
- Secondary headache related to ophthalmic pathology (e.g., uncorrected refractive error).

**PLAN:**

- Tests:
- Comprehensive ophthalmological evaluation, including visual acuity with pinhole, subjective refraction, slit lamp examination, and dilated funduscopy.
- Consider corneal topography if astigmatism or keratoconus is suspected.
- Medications:
- Ibuprofen 400-600 mg by mouth every 6 hours as needed for headache.
- Patient Advice:
- Avoid prolonged screen time or activities that may exacerbate eye strain.
- Monitor vision for any changes or worsening, and note any new symptoms.
- If symptoms significantly worsen or new neurological deficits develop, seek immediate medical attention.
- Follow-up Instructions:
- Urgent referral to ophthalmology for definitive diagnosis and management of monocular diplopia.
- Follow up with primary care in 1-2 weeks, or sooner if symptoms are unresponsive to pain management or worsen.